

The Careoperative

[Preamble](#)

[Section 1 - Guiding Principles](#)

[Article 1 -- Be Respectful](#)

[Article 2 -- Be Responsible](#)

[Article 3 -- Communicate Early and Often](#)

[Article 4 -- Be supportive](#)

[Article 5 -- Equity](#)

[Section 2 - Guiding Practice](#)

[Section 3 - Group Decision-making](#)

[Section 4 - Publication and Authorship](#)

[Courtesy Notice](#)

[Opt-in](#)

[Co-authoring with non Careoperative Participants](#)

[Duties of authorship](#)

[Order of authors](#)

[Rights and responsibilities of first author\(s\)](#)

[Dispute resolution](#)

[Recognition of power imbalances](#)

[Definitions related to authorship](#)

[Authorship](#)

[Contribution](#)

[Section 5 - Preventing and resolving conflicts](#)

[Steps that we have already taken in preventing conflicts:](#)

[Potential beneficial actions that we can further work on:](#)

[Protocol for resolving conflicts and complaints](#)

[In the case of conflict between two parties \(or two people\)](#)

[General reflections](#)

Preamble

The Careoperative is a leadership collective which we define as a group of individuals from multiple organisations and sectors who lead transformational social change together through

critical reflection, inclusivity and care (see Care et al. forthcoming). See also:
https://docs.google.com/document/d/1D_3xhDequZnuFU8xM9zLQY0uSukIEPjSLXru0dkdZko/edit?usp=sharing

We agree to abide by this Code of Collaboration for all matters relating to our shared activities as the Careoperative. This doesn't mean we haven't already been doing a good job :)

By following this code, we hope to create an experience that is beneficial, productive, and enjoyable for all involved; one that enables us to deepen our teamwork and leadership skills, and to develop as individuals and scholars.

This document is intended to live alongside our group - it will change and grow as we need it to. Everyone is welcome to suggest what it needs to say, or if parts of it don't work.

In our collaboration, we will strive to be involved/committed, honest, respectful, enthusiastic, adaptable, and supportive of each other and our ideas. We respect the confidentiality of reflections and stories shared. We aim to help each other, ourselves, the stakeholders we work with, our students, mentors, colleagues, and our undisciplined communities to create a safe and productive environment for collaboration.

We are professional researchers who are committed to ethical and fair research practices, and specific institutional codes and procedures. This does not supercede or replace these. Rather it seeks to address specific needs of working as a group of postdoctoral researchers embarking on inter- and trans-disciplinary collaboration. It also seeks to make clear some of the 'unwritten' expectations we have of each other and of ourselves.

This code is not intended to be overly directive or a set of rules to follow. It is rather about trying to establish shared understanding of what is most productive and safe for us as a group of colleagues. It also tries to pre-empt what can go wrong in these types of situations and help us to avoid uncomfortable or unsatisfactory experiences. The examples of how we might best work together are intended to guide this, and to provide a resource for the future as we go on to work in and manage larger transdisciplinary efforts. The Code might also prompt each of us to reflect on our contributions to the group, and what we can realistically commit to.

Section 1 - Guiding Principles

This section suggests the broad principles we aspire to when working together, including some examples of how we can enact these in our collaborations.

Article 1 -- Be Respectful

- Be respectful of others. Respect the needs, time, opinions, limits and concerns of your colleagues¹.
- Be respectful of yourself and of your needs; treat yourself well.
- Come prepared to virtual and face-to-face team-meetings. Even if you're not able to attend complete work prior to the meeting.
- Let colleagues know if you are unable to make a meeting or if you will be late - email the designated chair.
- Listen when individuals are speaking; don't interrupt; be honest. Remember that silence can be valuable too.
- Act respectfully during meetings, e.g. not making calls or texts unless urgent.
- Strive for "+1 policy" in which critique is coupled with constructive follow up. Also seek win-win solutions which have multiple benefits, or achieve more than one goal through a single action.
- Be respectful of differences and mindful that not all our differences are obvious. This includes remembering that there are many different academic disciplines, and many equally valid ways of doing research.
- Remember that you are under no pressure to participate in all group activity. If you do not want to consent to proposed group work you do not have to.
- If someone decides not to participate in an activity, respect their decision.
- Strive to respectfully tell someone if they are interrupting.
- Strive to avoid complaining. Instead use more productive methods of airing frustrations (e.g. mention what bothers you and why and what you think you can do about improve the situation; this might include asking for help from fellow Careoperative members or others).

Article 2 -- Be Responsible

- Be professional in interactions with stakeholders, colleagues, professors, and peers; you represent yourself, your institution and the Careoperative .
- Come to meetings prepared.
- Be ambitious, but don't promise what can't be delivered!
- Follow through on action items and other commitments when promised. If this is not possible that's fine. Aim to let another member know you're struggling and to pass the task on.
- Adhere to ethical research standards regarding respect, benevolence, confidentiality and privacy.
- Be pragmatic: make thoughtful, deliberate decisions; don't be hasty, but don't linger!

¹ By colleagues we mostly mean the fellow members of the Bosch Academy. But this principle will also apply to those we collaborate and interact with when working together.

Article 3 -- Communicate Early and Often

- Collaborate vocally to avoid misunderstandings.
- Understand that although we do our best to schedule in advance, urgent considerations do arise. If you can no longer join a meeting communicate as early as possible.
- Everyone is busy - endeavour to communicate what others need to know, without sharing too much too often.
- Strive to use good email etiquette (e.g. <https://www.kent.ac.uk/itservices/email/emailcharter.html>)
- Slack is our main and preferred communication channel, and should be kept up to date with activity. Other channels have specific uses:
 - Google Drive: to store and work with common documents
 - Email: urgent or time sensitive communication, which requires attention from everyone / specific subgroup members
 - Whatsapp: informal communication, includes those opting to join
 - Zoom: to hold group meetings.

Article 4 -- Be supportive

- Be supportive of colleagues as you work toward meeting project, learning, and personal objectives.
- Cooperate smartly: make use of your strengths and let others apply theirs. This way we can cover each other's weaknesses, and learn how to address them.
- When processes aren't working, acknowledge it, take time to reflect, evaluate the situation, and share suggestions for improvement.

Article 5 -- Equity

- Treat each other fairly and justly, whilst respecting our differences.
- Be mindful that not all our differences are obvious, and other people have needs different from yours.
- We believe that everyone has the right to be in a respectful, safe and welcoming working environment.
- We do not tolerate discrimination, harassment, bullying and behaviours that are patronising.
- Strive to promote academic standards which aim to prevent harassment and discrimination at events e.g. <https://www.rgs.org/research/annual-international-conference/at-the-conference/code-of-conduct/>

- We recognise that membership of the Careoperative occurred through a selection process we did not determine and which may not have been equitable or inclusive. We seek inclusivity to remedy the systematic marginalization of people (whether because of gender, ability, racial or class constructs, etc.) excluded by traditional leadership models.

Section 2 - Guiding Practice

Levels of participation

At the annual meeting, members select their level of participation (see table below).

	0 Sleeping member	<0.5 Observer	>0.5 Active member	Caretaker
Once a year check-in	😊	😊	😊	😊
Attendance of annual meeting		😊	😊	😊
Attendance of quarterly meeting			😊	😊
(Somewhat regular or selective) attendance of weekly meetings (e.g. attendance of only business or fun meetings)			😊	😊
Caretaker meetings				😊
Involvement in decision-making (asked to take part in decision-making process, who to ask?)				
Possibility to provide input for a decision via “bridge person” in case one cannot attend a meeting in which a decision will be taken	😊	😊	😊	😊
Strategic decisions (affecting the long term life of the Careoperative)	😊	😊	😊	😊
Strategic decisions affecting specific projects and outputs (e.g. paper)		😊	😊	😊
Logistic decisions		😊	😊	😊
Check-ins with others (reach out actively to others)			😊	😊

Take care that these levels are reviewed annually				😊
---	--	--	--	---

Meetings

- Every meeting (physical and online) should have a facilitator. Anyone can volunteer to facilitate a meeting. Ideally, facilitation will rotate between members to share the responsibility and opportunity.
- Every meeting (physical and online) should have a note taker. Anyone can volunteer to take notes at a meeting. Ideally, note-taking will rotate between members to share the responsibility and opportunity. Notes should be shared in the Google Drive:
<https://docs.google.com/spreadsheets/d/1HMMR3aMMxygcbBzLLqkbGkRVcFJr1-qMnOz4HgFvpSs/edit?usp=sharing>
- Every meeting should have an agenda. Anyone can propose agenda items via the shared Google Sheet.
- Every meeting should be as concise as possible. Ideally online meetings will last 60 minutes.
- The facilitator is responsible for time-keeping.
- If longer than this a break time should be included.
- The meeting facilitator will help the group reach decisions through a fair process which aims for consensus.
- Where a meeting requires focused activity or detailed discussion of details we will use break-out groups. A typical process for forming and working as groups will be:
 1. Time for whole group to surface and cluster issues, guided by facilitator
 2. Time for people to volunteer to host and/or participate in break-out conversations
 3. 5 minutes to disperse to breakout groups, including comfort break and a 'fun' task
 4. 20 - 30 minutes discussion and for each breakout group to prepare a "proposal for the group"
 5. 5 minutes for comfort and to reconvene
 6. 20 - 30 minute reporting proposals to whole group
 7. Opportunity for clarifying questions and discussion
 8. Indicate approval (e.g. thumbs up/down). Follow sociocratic decision making process if necessary.
 9. Celebrate!

Section 3 - Group Decision-making

Group decision-making is adaptable to each setting and situation that our group may be in. Part of that entails recognizing when a shift in group situations (i.e. moving from formal to informal workshop settings) occurs that needs decision-making.

The facilitator can choose the decision-making style that fits best to them and the situation. There are a number of decision-making styles and basic principles (excluding autocratic/directive) that are available to the facilitators for supporting decision-making. Each facilitator can implement decision-making processes as they wish, given they are prepared to do so. Some ideas and models for decision-making include, but are certainly not limited to:

- **Overt objection**
 - o E.g. consensus seems to say xyz, does anyone have any objection?
- **Covert objection**
 - o Facilitator tables a decision and makes eye contact/nod with everyone and if there is not unanimous approval, reopens the conversation
- **Sociocratic consent** (e.g. thesociocracygroup.com/home/basic-principles/4-principles/)
 - o The principle of consent governs decision-making. Consent means each participant has no fundamental objections to a proposed decision.
 - o An objection is not a veto; it is a valid reason why a particular decision will prevent a participant from doing their job or otherwise supporting the group's aims. Objections are asked for because they provide positive information.
 - o Consent and objection are negotiated in the communication between sub-groups and whole group. The whole group has a facilitator, and the whole group is split into sub-groups that each designates a facilitator. Sub-groups make decisions, and these are communicated to the whole group. Decision-making process can travel between sub-groups and larger group facilitators, in each direction, in several iterations, until consent is achieved.
- **Systematic consensus** (e.g. www.seedsforchange.org.uk/shortconsensus)
 - o Consensus is neither compromise nor unanimity - it aims to integrate everyone's best ideas and most important concerns.
 - o The first step is for the group participants to gather as many possible ideas as possible.
 - o The proposed ideas are evaluated and judged by each participant. Each participant gives each idea anywhere from 0 to 10 points. 0 points is complete acceptance of the idea, 10 points is the maximum objection to the idea. The points are then added together to calculate where there is the maximum and minimum objections to each of the ideas. The proposal with the minimum objection is the idea which comes nearest to consensus.

- In addition to the ideas, there is also the “zero option.” This includes options such as “leave everything the way it is,” “do nothing,” “decide the next time.” This helps discern whether an idea has a chance of being accepted by the group.

Within each of the different settings, group decision-making also may be needed to move through impasses. This can be done individually, in sub-groups, or as a whole group. Steps could include finding common ground on a next step (e.g. postpone decision X and decide on Y) or implement a different decision making method (e.g. Think-Pair-Share: brainstorm individually for 5 minutes, write down proposal, share with the group, and then engage with the other participants on the ideas.

Verbal and written silence will be considered acquiescence if notice for review (e.g. of a group document) has been given.

Our typical decision making process is:

1. Proposal - person/representative of sub-group summarises key elements and justification.
2. Clarifications - open round inviting clarifying questions.
3. Amendments - amended/counter proposal made if required.
4. Indicate agreement - thumbs-up / thumbs-down to express agreement / disagreement.
5. Explain objections - disagreeing participants express views.
6. Decision:
 - a. Consensus - proposal agreed
 - b. Slight disagreement - proposal accepted as “good enough for now, safe enough to try”.
 - c. Substantial disagreement - proposal needs more work, goes onto future agendas.
 - d. Follow-up - Clear responsibilities for tasks distributed.

If time is short or we need to move on the group can be asked to endorse a decision as GEFNSETT: “good enough for now, safe enough to try”.

The decision process is articulated via the Decision Tree (see Annex I)

Section 4 - Publication and Authorship

Recognizing that our Careoperative is reflective and future work will draw upon the efforts of a large number of participants; that the interests and incentives to publish in peer-reviewed academic journals of these participants are diverse; and that the question of authorship, if not worked out in advance through a fair and inclusive process, can generate undesired tensions within the group, we share a proposal for a Careoperative publication and authorship guidance

document revolving around the obligation of courtesy notice; the requirement to offer opt-in; and clear expectations of duties of authorship.

Courtesy Notice

Any participant seeking to develop a manuscript for publication where credit to others' work is due, drawing on the intellectual contribution of other participants, is strongly encouraged to provide written notice to all participants of an intent to develop said paper.

- Notice must be provided in writing (e.g., email), and as early in the process as possible, preferably at the stage of conceptualization and target journal selection.
- Participants must have an equivalent of 10 working days to respond to a courtesy notice of publication, for example expressing a desire to be more actively included, or not.

An interest in pursuing single authored works DOES NOT exempt participants from this guidance on publication and authorship, especially as the intellectual contributions of others may be implicated.

Opt-in

In case a participant expresses interest in taking part and contributing to the process of writing up a manuscript for a peer-reviewed publication or otherwise, he/she accepts the Duties of Authorship agreed upon and elaborated below.

- A participant's decision to opt-in must be sent in writing (to the sender of notice) within the above-mentioned 10-working-day-equivalent period (e.g., not counting days when participants are on holiday or significant travel)
- The member giving notice must acknowledge receipt of desire to opt-in
- The member giving notice must then include opted-in co-authors in all forward written, virtual, and physical communication regarding said manuscript

If a consortium member has contributed to the input for the proposed publication, but decides to OPT-OUT of publication, her/his contributions will be duly acknowledged in the form of manuscript citations, footnotes, and/or acknowledgements, as appropriate to the final form of the manuscript.

Co-authoring with non Careoperative Participants

Any participant interested in co-authoring with non-Careoperative participant on careoperative related activities, is welcome to do so but is still responsible for ensuring said non-Careoperative participant is aware of our guidance on authorship and publication. Said participant is responsible for verifying this awareness and supporting said non-Careoperative-er in

compliance. In turn said non-Careoperative participant can expect to receive equal respect and treatment by all participants in the Academy.

Duties of authorship

Participants who “opt-in” accept and agree to honor the DUTIES OF AUTHORSHIP, elaborated below. The duties of authorship in the group are in keeping with the The Netherlands Code of Conduct for Research Integrity (2018)

(<http://www.vsnul.nl/files/documents/Netherlands%20Code%20of%20Conduct%20for%20Research%20Integrity%202018.pdf>), a specification of the general European Code of Conduct for Research Integrity (e.g., section 2.6, collaborative working)

(https://ec.europa.eu/research/participants/data/ref/h2020/other/hi/h2020-ethics_code-of-conduct_en.pdf) and consist of duties to:

- **Include:** the lead / instigating author must include authors who have designated the desire to opt-in;
- **Communicate:** respond to emails requesting feedback or scheduling author team meetings;
- **Participate:** attend calls, virtual, or in-person meetings related to the further conceptualization, writing, and revision of a given writing project;
- **Write:** contribute substantial research and writing to specific sections of the manuscript, as agreed upon by the writing team or reasonably delegated by the lead author(s);
 - Making substantial and constructive contribution throughout the development of the manuscript—for example, comments that not only offer ideas, praise, or provide criticism, but also actively provide alternative text, references, etc. to progress the manuscript

If these duties are not met in good faith, then the authorship of the manuscript may be subject to review and change. In the case of disagreements on authorship, disputes may be resolved in mediation by consortium members or external contributors, as defined below.

Order of authors

First authorship and remaining co-authorship order should be agreed upon as early as possible in the manuscript-drafting process (with due consideration of any PhD projects related to the manuscript). In most cases the first author is envisioned as the initiator of the notice for publication, however, this may not always be nor always remain the case. Alterations to the order of authorship should be agreed upon by all the authors as a consensus agreement. Such agreement should be documented in writing (e.g. email).

Where equal effort of a set of co-lead authors is envisioned, alphabetical author with * indicating equal contribution of co-leads should be pursued.

Rights and responsibilities of first author(s)

The first author(s) are responsible for the accuracy and content of the whole publication from the initiation of the drafting process to the successful publication in a journal/book. Therefore, if a consensus cannot be reached during the drafting process, the first author(s) will have final decision authority regarding content, authorship and co-author positions, journal selection, and acknowledgements.

Dispute resolution

In the event that opting-in authors do not honor the duties of authorship; or first-authors do not honor the duties of authorship; or any co-author does not honor the duties of co-authorship, it will be considered grounds for removal of said author from the author team.

Dispute resolution processes, as elaborated in this code of collaboration, will also be used in the case of disagreements over intent to pursue single-authored works.

Recognition of power imbalances

We recognize that in some cases, senior researchers may co-author with junior researchers; participants with project leadership roles elsewhere (e.g., coordinator or Work Package leader) may co-author with others in non-leadership positions. These seniority- and role- differences may generate power imbalances and create tensions within author teams. For example, junior researchers may feel uncomfortable questioning a senior co-author; a non- leader member might fear retaliation from a member in a leadership position. In such cases, independent participants of the Bosch cohort or an external Publication Ethics Review may be pursued to support communication and dispute resolution among the author team.

Definitions related to authorship

For more elaborate definitions of authorship and guidance on author orders, readers are referred to international standards, abridged below, for convenience.

Authorship

We adopt the International Committee of Publication Ethics (COPE) definition of authorship (https://publicationethics.org/files/2003pdf12_0.pdf), as follows:

“Authorship credit should be based only on:

(1) substantial contributions to conception and design, or acquisition of data, or analysis and interpretation of data;

(2) drafting the article or revising it critically for important intellectual content; and

(3) final approval of the version to be published.

Conditions (1), (2), and (3) must all be met. Acquisition of funding, the collection of data, or general supervision of the research group, by themselves, do not justify authorship.”

Contribution

We adopt the position of the International Committee of Medical Journal Editors in the recommendation that authors should provide a description of what each contributed (<http://www.icmje.org/recommendations/browse/roles-and-responsibilities/defining-the-role-of-authors-and-contributors.html>). The US National Academies of Science Engineering and Medicine, Fostering Integrity in Research report (<https://www.nap.edu/download/21896>, pages 156-157) offers further elaboration on definitions of contribution with regards to authorship:

“Standards should specify the appropriate roles that merit designation as an author:

- Substantial intellectual contribution to conceiving, designing, or planning the research to be reported;
- Substantial intellectual contribution to acquiring, analyzing, or interpreting the primary data;
- Substantial intellectual contribution to drafting or revising the article reporting the research in question.

Standards should specify the contributions that do not merit authorship but may merit acknowledgment and/or citation:

- Securing funding for the research;
- Providing general supervisory or administrative support for the research;
- Technical writing, editing, and proofreading of the article reporting the research;
- Making available data collected for previously reported work, or providing materials or specimens.”

The Netherlands Code of Conduct for Research Integrity (2018)

(<http://www.vsnu.nl/files/documents/Netherlands%20Code%20of%20Conduct%20for%20Research%20Integrity%202018.pdf>), a specification of the European Code of Conduct for Research Integrity, provides an additional source of guidance on the question of contribution.

Section 5 - Preventing and resolving conflicts

Disagreements, tensions and conflicts are unavoidable as part of any collaborative endeavour. These are needed to recognize how we are different and to appreciate the diversity within the group and are an opportunity for emotional learning and developing assertivity. Aware of this, we are working towards building foundations for a supporting and caring atmosphere of collaboration. An important aspect for that is to have a guiding strategy that can help us to prevent difficulties or conflicts in the first place and, in the case they arise, to mediate and to facilitate understanding and reconciliation.

Steps that we have already taken in preventing conflicts:

- Developed a common vision.
- Promoted collaboration and avoided competitive attitudes through this code of collaboration and our expressed commitment to acting as Care Support for each other.
- Established a decision-making procedure, that we need to make sure is properly used
- Identified the different roles within the group (disclosing/displaying, sharing and rotating those) in moderating, note taking, floor taking.
- Elected Caretakers for one year.
- Established processes for reviewing our work and ways of working together (e.g. Quarterly and annual meetings).

Potential beneficial actions that we can further work on:

- Create spaces for joint celebration, moments when the whole group can feel and connect with the unity that underlies and supports existing diversity, beyond visible differences, whether mental or emotional.
- Create a space for collective emotional management. There are some techniques such as the Forum ZEGG or the Process Oriented Forum (Mindell 2002).
- Create a committee of mediators for conflict resolution: The committee will be the group to get in contact in case a difficulty or conflict arise and there is a need of mediation. The committee will be in charge of identifying the type of conflict, talking with the parts involved and try to mediate a process for reconciling the parts. Once the committee has been formed. This may involve to: a) Create a space of trust and security, first working with parts separately and then jointly, b) clarify perceptions, misunderstandings and divergent viewpoints, c) distinguish between positions, interests and needs, d) identify minor agreements that can easily work out, e) develop oral or written joint agreements

with responsibilities of each party (see specific steps for conflict resolution on the protocol suggested below).

- Support personal work on interpersonal and group communication as well as learning to know the roles that can be identified from group members.
- Implement a training on Nonviolent communication for all members of the *Careoperative*. Some resources: see Rosenberg, M., & Chopra, D. (2015). *Nonviolent communication: A language of life: Life-changing tools for healthy relationships*. PuddleDancer Press. See also: <https://www.youtube.com/watch?v=VT8KGgDo6TY>
- A principle when we identify/feel in conflict: Be open, don't hide it, and never let the conflict escalate.

Protocol for resolving conflicts and complaints

Before starting with the steps of the protocol, some examples about the typical phases of conflicts are provided. This scheme is intended to ease the identification of the beginning of conflicts and to deal with them before it is too late:

1. **Discomfort:** Intuitive feeling that something is wrong, with someone, with the group, with your life. It's time to ask yourself: Is there anything I could do now about it?
2. **Incidents:** Something unimportant that leaves us feeling worried or certain irritation, usually for a short time, so we tend to forget it.
3. **Misunderstandings:** When our expectations of how things are meant to be, based on assumptions not contrasted with others assumptions, are not met. Usually happens when communication is poor or unclear. Or because we don't want to see that others are different than we would like them to be.
4. **Tension:** It is already a permanent state that accompanies our interactions with other people. It manifests itself through an abundance of negative actions, of opinions and behaviors, which with more or less awareness, in a more or less subtle way, are intended to harm.
5. **Crisis:** The conflict becomes apparent, and repressed emotions until then are released with force, passion, and even violence can appear.

In the case of conflict between two parties (or two people)

The concrete steps of the protocol will be:

1. Identification and acknowledgement of the conflict (awareness of what happened, which is recognized by the parties)
 2. Check of the willingness to resolve the conflict (this would be a group agreement)
 3. Attempt to resolve by the parties involved (If it is not possible, go to point 4).
- Tools:** A script to guide the parties involved in the resolution of the conflict.
- a. Collaborate in creating an appropriate atmosphere, encourage empathic communication

- b. Clarify perceptions: how do I see the conflict? How does the other person see it? Where do our differences get stuck?
 - c. Distinguish between positions (what we say), interests (what we want to achieve) and needs, between individual and shared needs
 - d. Managing emotions: "How to manage my emotions? How do others handle them? What can I do to have a better use of my emotions?"
 - e. Build a positive shared power (or agency): what is my power (or agency)? What power (agency) does the other party have? How can we use our power (agency) together for mutual benefit?
 - f. Reconciliation and forgiveness: how did the past influence the relationship? Do I have a need to apologize? Should I apologize?
 - g. Generic options and establish agreements
 - h. Cooperate in its realization
4. If step 3 is not possible or has not generated a solution suitable for both parties, the committee for conflict resolution will be contacted. A suitable person will be sought for the facilitation by mutual agreement (if it is an internal person, would continue with the protocol, and if it is external to the *Careoperative* the conflict committee will act as an interlocutor with this person to accompany the process)

- a) In case the committee is involved in the conflict, contact an "impartial" third person.

Steps:

4.1 Information: This person will speak with the parties involved separately to obtain the necessary information. The facilitator will try to follow the point scheme above to clarify perceptions, emotions, needs of both parties, facilitating communication in a non-violent way and helping them to recover their responsibility in the conflict. The facilitator explains the process to follow and consent is requested to start with the mediation. Many conflicts endure because on their own existence the parties involved justify their life. Ask: How do you want your life to be when this is over?

4.2. Putting in common: once the information step has been developed the two people will meet with the committee and will share emotions, needs and individual responsibilities within the conflict. From this point onwards, work will continue with both parties.

4.3. Change of roles: Later, each party will perform an exercise of putting themselves in the place of the other party and try to describe their feelings and needs.

4.4. Problem framing: the facilitator will propose a framing of the conflict or problem asking both parties whether they recognize themselves in this framing. In this part the conflicting knots, and points of divergence and convergence. It is about establishing a common understanding of the most important issues that have to be solved.

4.5. Generate viable solutions. Brainstorm with green hat (related to lateral thinking, i.e. focusing on creativity; possibilities, alternatives and new ideas. For more on this see the Bono group - six thinking hats).

4.6. Choose one mutually accepted. Each of the solutions will be valued until finding the most appropriate.

4.7. Plan the implementation (and the review if it is necessary).

4.8. Plan the evaluation.

Tools for the committee. 4 fundamental strategies to resolve a conflict:

- a. Containment: it helps us put the conflict under control, especially when a situation of chaos has been reached. It can be static or dynamic.
- b. Confrontation: necessary in latent conflicts that nobody dares to face openly
- c. Compassion: if one or both parties feel shattered the best that you can do is actively listen to your story without trying to interpret it or look for answers.
- d. Collaboration: in the end it is necessary to transcend the conflict, compromise to the parties in a creative relationship.

5. Resolution in an extraordinary meeting. The group will serve as a resonant of the emotions. Here it is important to work with the whole group, what are the emotions that appear, what roles are given in each of the members of the Careoperative regarding the conflict. A scheme similar to that of point 4 will be followed looking for emotions and needs and identifying conflicting knots to make the framing. Collective solutions will be generated and one of them will be implemented planning the implementation (where possibly tasks will be also performed by people who have not been involved in the conflict). At the same time, a planning the evaluation of the executed alternative will be developed. The facilitation will be carried out by the committee in case they have not been part of the conflict. In case of being considered adequate, it will be possible to facilitate through techniques for conflict management such as FORUM.

6. Search for facilitation outside the careoperative. The committee of mediators will be in charge to search for an external facilitator and to inform the rest of the team about the person who would come and about the process to follow, the method, etc. The rest of the members must be in agreement with the facilitator and facilitation method proposed.

General reflections

In all cases, a small reflection on the context of the conflict would be interesting to try to discern what is happening in the group that is favoring the development of this conflict and assess the need to search for alternatives at the systemic level. This would be done in a whole group meeting. In case the conflict does not reach this level, the committee will be in charge of

carrying out this reflection and will share with the whole team their assessment in order to enrich and implement the appropriate measures if any.

In case of a conflict encompassing the whole group, it would go directly to the resolution step 5 (in extraordinary meeting) and if it is not enough it would go to step 6.

ANNEX I. Decision-making tree

